CONSULT FOR AFRICA
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Building the IVF business

A nine month mandate to grow Clearview Fertility: what we would build, what you would own at the end of it, how it is measured, and what it costs.

Dr Kunle Ajayi

Dr Kunle, thank you for the conversation and for the time you have given this. What follows is the proposal I said I would write, and it is deliberately narrower than the strategy note. That note covered the whole group. This covers one thing, which is building the fertility business, because that is where the money is and because it is the part you asked me about.

I want to be straightforward about the argument, because it is the same one I made when we spoke and it has not softened. Growing an IVF business is not mainly a question of finding more patients. It is a question of four engines, and only one of them involves a stranger who has never heard of you. The other three are conversion, which is turning the people who already contact you into people who cycle, retention, which is the frozen transfers and the second child, and capture, which is making sure the pregnancy you created is looked after and delivered inside your own group rather than somebody else's. Those three run on demand you already generate and already pay for, so they are cheaper, faster and considerably more certain than advertising. We build those first. Then we open the catchment, which is the fourth engine and the one with the most room in it.

The programme is nine months in three phases, and I have structured it so that you are never committed further than you have seen. The first phase is four weeks and it ends with a decision. If what we find does not support the rest, you stop there and you will have paid for four weeks, and you will still own a counted funnel and your own live birth rate by age band, which is worth having whatever you decide to do next.

The five surveys are already live at consultforafrica.com/ClearviewProject, and I would like them running while we agree the rest, because the one for people who enquired and did not go ahead takes weeks to gather and it is the most useful thing on that page. Nothing on it commits you to anything.

On the fee I have shown you the method rather than a headline. Section 06 sets out the grades, the published day rates and the number of days each piece of work actually takes, so you can see exactly what you are buying. You are paying for effort rather than a share of anything, which also means that if a workstream turns out not to be needed it comes straight off. I have tied the fee to measures you can hold me to, and if we miss them you withhold the tranche.

The natural first step is to start Phase 1 while the surveys gather. I am ready when you are.

Warm regards,

Dr Debo Odulana
Founding Partner, Consult for Africa

01 / THE OBJECTIVE

What this mandate is for.

	Objective
1	Grow Clearview Fertility without first spending money on acquisition, by closing the three leaks that sit inside demand you already generate.
2	Make the fertility business legible: a counted funnel, a live birth rate by age band, a known laboratory ceiling, and a monthly pack that tells you what is happening.
3	Capture the whole journey inside the group, so the pregnancy Clearview Fertility creates is booked, delivered and followed up at Clearview Hospital.
4	Open the catchment beyond Lekki, so the laboratory is fed by mainland Lagos, the rest of Nigeria, West Africa and the diaspora rather than by six kilometres of the most contested market in the country.
5	Build all of it so that it keeps working in a week when you are not in the building, which is what you actually asked me for.

What this mandate is not. It is not a marketing retainer and it does not include media spend. We do not advertise a business that loses nine enquiries in ten, because that makes the loss bigger rather than the business. Acquisition comes after the machine that catches it works, and by then it is a much smaller bill.

02 / THE FOUR ENGINES

Where IVF growth actually comes from, in order of cost.

The figures below are calibrated on the assumptions in the strategy note, at 15 cycles a month and an all in price around NGN 4.8 million. They will move once Phase 1 counts them, which is the point of Phase 1.

	Engine	What it is	What it is worth	Cost to build
1	Conversion	Roughly 120 people a month already contact you and around nine in ten never cycle. Five points of conversion is about 6 more cycles a month on enquiries you already pay for.	NGN 28.5 m a month	Lowest
2	Retention	A working recall turns the tank into about 4 frozen transfers a month at the best margin in the building, plus annual storage you are currently not billing.	NGN 11.4 m a month	Low
3	Capture	About 62 ongoing pregnancies and 37 caesarean sections a year, which you create. The only question is whose hospital bills them.	NGN 139 m a year	Low
4	Inbound	Four additional cycles a month from outside Lagos, on a laboratory already built and already paid for. Slowest to build and the only one with real headroom.	NGN 262 to 365 m a year	Highest

The order matters more than any single item on that list. Engines one, two and three run on demand that already exists, so they pay for engine four. Almost every fertility clinic in this market does it the other way round, spends a year of profit on acquisition, and finishes the year with the same number of cycles and a thinner margin.

03 / THE PROGRAMME

Three phases over nine months, with a decision after the first.

	Phase	What it does, and the workstreams in it	When	Days
1	Baseline	Establish what is actually true before anything is built. **A.** Count the funnel, and pull the outcomes **B.** The tank: storage, consent and the recall list **C.** Ten patients traced end to end, across both companies **D.** The five surveys, the site visit and the findings session	Weeks 1 to 4	64
2	The conversion and continuity build	Turn demand you already generate into cycles, and keep what you win. **E.** The enquiry pathway, and the coordinator function **F.** Packages, pricing, and how a family actually pays **G.** The recall and sibling programme **H.** Downstream capture into Clearview Hospital **I.** Outcomes reporting, and what may honestly be claimed	Months 2 to 6	164
3	The inbound engine	Open the catchment beyond Lekki, and make the laboratory carry it. **J.** The traveller proposition and the package **K.** The shared care network: recruit, contract, protocol **L.** Laboratory depth and the capacity ceiling **M.** Running the programme, and the monthly pack	Months 4 to 9	142

The decision at the end of Phase 1

Phase 1 is four weeks and it stands on its own. At the end of it you will know your conversion, your live birth rate by age band, what is in the tank, where your pregnancies deliver and what your laboratory could carry. If that evidence does not support the rest of the programme, we stop and you have paid for four weeks. I would rather find that out in month one than have you discover it in month six, and a mandate that cannot survive its own baseline should not be sold to you in the first place.

04 / WHAT YOU OWN AT THE END

Working systems, not a report.

What	What it actually is
The enquiry register and the pathway	Every enquiry logged, a named coordinator role with a written job description and the person recruited into it, a same day response standard, and a follow up cadence that runs whether or not anybody remembers.
The price book and the packages	A written, costed price list a patient can be sent the same day. Multi cycle and journey packages, an instalment structure, and a repricing schedule tied to the exchange rate so the currency stops setting your margin.
Your own outcome data	Live birth rate by age band, from your own records, in a form you can put in front of a patient without flinching, and the rule for what may and may not be claimed from it.
The recall and sibling programme	Every patient with embryos identified, the consent and disposal position settled, storage billed, and a standing recall that runs on a calendar.
The continuity pathway	A written route from positive pregnancy test into Clearview Hospital, booked by default, with a shared antenatal protocol for patients who live elsewhere.
The shared care network	Named partner gynaecologists under written agreement, paid for monitoring work rather than for referrals, with the protocol, the fee schedule and the clinical governance behind it.
The traveller package	One price covering treatment, accommodation, transfers, coordination, and admission to Clearview Hospital if a complication of the cycle requires it.
The monthly pack	One page a month: cycles, conversion, outcomes, the tank, the pregnancies, the money. Yours, and it keeps running after we leave.

05 / HOW IT IS MEASURED

What each phase has to put on the table.

I would rather be held to something than ask you to take this on trust. Each phase carries a measure, and the final tranche of that phase is not invoiced until the measure is met.

	Phase	What has to be true before the final tranche is invoiced
1	Baseline	A counted funnel, a live birth rate by age band, and a capacity ceiling for the laboratory, all in writing.
2	The conversion and continuity build	Enquiry to cycle conversion improved by at least five percentage points on the Phase 1 baseline, and every IVF pregnancy offered a booking at Clearview Hospital by default.
3	The inbound engine	At least six partner gynaecologists contracted and running monitoring under protocol, and the first travelling patients treated on the package.

06 / WHAT IT COSTS

Resourced time. Grades, published rates, and the days each piece takes.

You are buying effort, not a percentage of anything. Every line below is a number of days at a published rate, which means you can see what you are paying for, you can challenge any line, and if a workstream is dropped it comes straight off the fee.

	Workstream	Days	NGN m
A	Count the funnel, and pull the outcomes	25	6.7
B	The tank: storage, consent and the recall list	11	2.2
C	Ten patients traced end to end, across both companies	14	4.4
D	The five surveys, the site visit and the findings session	14	4.7
E	The enquiry pathway, and the coordinator function	43	14.6
F	Packages, pricing, and how a family actually pays	32	11.8
G	The recall and sibling programme	26	7.6
H	Downstream capture into Clearview Hospital	34	12.8
I	Outcomes reporting, and what may honestly be claimed	29	9.8
J	The traveller proposition and the package	27	9.9
K	The shared care network: recruit, contract, protocol	58	20.0
L	Laboratory depth and the capacity ceiling	19	8.0
M	Running the programme, and the monthly pack	38	13.8

The same money, by grade

Grade	Day rate	Days	NGN m
Partner	750,000	40	30.0
Principal	450,000	84	37.8
Senior consultant	300,000	157	47.1
Consultant	190,000	10	1.9
Analyst	120,000	79	9.5
Gross		**370**	**126.3**

	NGN m
Gross, at published rates	126.3
Programme rate, because this is one continuous mandate rather than five engagements, at 10 per cent	(12.6)
Net fee, for the whole nine months	**113.7**

That is NGN 12.6 million a month across a nine month programme, against a business the strategy note puts at roughly NGN 71 million a month of fertility revenue today.

07 / TERMS

How it is invoiced, and what is not in it.

	How it works
On signature	A mobilisation payment of NGN 10.0 million, credited in full against the first invoices. It is not an additional charge.
Thereafter	Monthly in arrears against timesheets, capped at the net fee above. You never pay for a day that was not worked, and the cap does not move because we were slower than we expected to be.
The Phase 1 gate	At four weeks you may stop. If you do, you pay for Phase 1 and nothing further, and you keep everything it produced.
Scope changes	Any workstream may be dropped, and its days come off the fee. Anything added is quoted the same way before it starts.
Third party costs	Legal drafting, any software you choose to license, and any clinical or laboratory specialist we bring in are passed through at cost. We do not mark them up.
Not included	Media and advertising spend, equipment, laboratory consumables, recruitment salaries, and regulatory fees. Where we recruit for you, the fee for that is quoted separately and only if you ask for it.
Running it afterwards	If, at the end of the nine months, you would rather have an operator than an adviser, that is a different arrangement on a separate management fee. Running a business for years cannot sensibly be billed by the day. It is not part of this proposal and nothing here commits you to it.
Confidentiality	Everything we see is confidential to Consult for Africa. Patient level data stays inside your systems, and the surveys collect no patient identifiers at all.

08 / WHAT WE NEED FROM YOU

Not much, and most of it is access rather than work.

	What
1	A week of your own time across the nine months, most of it in the first month.
2	Access to the cycle register, the laboratory records, the storage log and twelve months of management accounts across both companies.
3	One person on your side who owns this with us. They do not have to be senior, but they do have to be in the building every day.
4	Permission to speak to your team, to a number of your patients with their consent, and to two or three of the doctors who refer to you.
5	The surveys pushed out properly. They cost nothing and they answer questions no amount of our time can.

The first thirty days. Start the five surveys this week, because the one for people who enquired and did not go ahead needs time to gather. Begin the count on day one. Pull the outcomes. Open the tank. Trace ten patients end to end. Then the two of us sit down with what it says and decide whether the rest of this programme is worth your money, which is a question neither of us can honestly answer today.

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